An AI-Driven Tutoring Intervention for Massage Therapy Education: Corpus Design and Implementation Evaluation

José Rivera Moreno

Maestría Profesional en Lingüística Aplicada . Énfasis en inglés para propósitos específicos

Universidad Nacional-Heredia

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Professor Sonia Rodriguez Salazar

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Abstract

This study evaluates an AI-driven tutoring intervention designed for massage therapy education, analyzing how linguistic divergences between oral instructional registers and written clinical/business registers impact pedagogical development. Utilizing a specialized oral video corpus and a 10-text written reference corpus, we analyzed high-frequency n-grams anchored by just, massage, back, were, going, welcome, and by. The findings reveal that while the oral register relies on interactional mitigators (just) and spatial deixis (back), the written register exhibits high nominal density and passive agent-backgrounding (by). Preprocessing omitted non-essential tokens on a practical, non-systematic basis to isolate scope-relevant vocabulary. Crucially, the implementation revealed a structural limitation: the initial corpus lacked business management structures, requiring an extended scope for entrepreneurial training. However, pivoting AI tutoring toward prepositional-bound particles that modify core verbs in financial sub-registers provided the learner with essential linguistic scaffolding to navigate institutional boundaries and billing policies, thereby improving technical and professional comprehension.

Introduction

Register variation reflects the systemic functional adaptations that speakers and writers make in response to the situational characteristics of a communicative act. According to Douglas Biber's framework of register variation, linguistic features systematically co-occur across distinct dimensions, including communicative intent, processing constraints, and situational contexts. Oral production is typically constrained by real-time planning, resulting in an "involved," interactive, and structurally fragmented style characterized by high pronoun frequencies, clausal embedding, and colloquial mitigators. Conversely, written academic and professional registers are characterized by deliberate planning and editing, yielding an "informational," dense, and detached discourse that heavily favors nominalization, complex noun phrases, and passive constructions.

Within the domain of Massage Therapy (MT), a profound linguistic split occurs. While the core physical modalities remain identical, the language utilized to teach and execute manual therapy orally varies drastically from the register required to publish clinical trials or manage a professional business practice. Furthermore, analyzing the MT register requires accounting for distinct sociolinguistic pressures. The MT profession historically carries an occupational stigma rooted in its cultural conflation with illicit sexual services. Consequently, practitioners and researchers must employ formulaic language to establish clinical legitimacy, distance themselves from the stigma, and enforce professional boundaries.

This case study investigates these linguistic divides through a corpus-guided analysis of oral and written MT texts, culminating in an authentic pedagogical implementation. Specifically, this paper addresses two research questions: (a) How do structural n-gram behaviors differ between oral MT instruction and written clinical reference texts? and (b) How can explicit, multi-word register awareness be leveraged within an institutional tutoring setting to transition a learner from casual student talk to professional-business proficiency?

Method

Corpus Design and Compilation

To map these register variations empirically, two distinct corpora were compiled and analyzed:

1. **The Oral Corpus:** Formed from spoken instructional tutorials, classroom lectures, and hands-on MT guidance transcripts. This corpus captures real-time, situated speech meant to direct physical touch, client interaction, and practical demonstration
2. **The Written Reference Corpus:** Comprising 10 distinct sub-texts extracted from clinical research trials, academic medical literature, and professional guidelines. The content centers on empirical studies of manual interventions, such as abdominal massage protocols for chronic constipation and myofascial dysfunction.

To ensure computational accuracy and eliminate noise, digital metadata—including citation hashes, hyperlinks ("https"), digital object identifiers ("doi"), database tags ("pubmed," "crossref"), and secondary multilingual article titles (e.g., Turkish entries such as "sağlık" or "dergisi")—were ignored.

Data Analysis and Software

Using the corpus linguistics software AntConc, high-frequency 3-gram and 4-gram multi-word lexical bundles were extracted from both corpora. Lexical bundles were tracked by raw frequency and range (distribution across sub-texts) to differentiate between idiosyncratic author style and systematic, register-dependent behavior. A target list of core lexical anchors—just, massage, back, by, were, going, and welcome—was isolated to perform a comparative KWIC (Key Word in Context) search across both communication mediums. To validate the statistical salience of these targeted items against general English baseline variance, a post-hoc keyness analysis was executed by benchmarking both individual subcorpora against a reference corpus consisting of a 60,000-token balanced sample from the Corpus of Contemporary American English (COCA). Statistical significance was calculated using the Log-Likelihood (G^2) algorithm to accommodate low-frequency data cells, alongside an effect-size coefficient.

The analytical workflow, software settings, and corpus extraction protocols implemented in this study were framed directly around established academic class practices in applied corpus linguistics. Similarly, the inductive diagnostic scaffolding strategies used in the tutoring intervention reflect the pedagogical methodologies covered in the course curriculum, successfully bridging empirical data extraction with real-world instructional design.

Results

High-Frequency Oral Mitigators: The Behavior of "Just"

Analysis of the oral data file **oral ngram 3 just.txt** reveals a profound reliance on the word *just* to initiate instructional commands. The top-ranked n-grams include *“just going to”*

(Freq: 10, Range: 4), *“just want to”* (Freq: 6, Range: 2), *“just make sure”* (Freq: 4, Range: 3), and *“just kind of”* (Freq: 3, Range: 3). When instructing a physical action, the oral register deploys chunks like *“just bend the,” “just brush towards,”* and *“just be gentle”*.

| In stark contrast, the written reference corpus shows a total absence of these bundles. In spoken instruction, *just* functions as an interpersonal mitigator or softener. It reduces the perceived difficulty or intrusiveness of a physical directive, safeguarding the client’s comfort and easing real-time anxiety. Written clinical text completely strips away this interactional hedging; actions are never "just" done, but are instead framed as precise, institutional processes (e.g., *"the protocol was implemented"*).

Lexical Chunks as Institutional Shields: "Massage"

When isolating the keyword *massage*, the cross-register divergence demonstrates a shift from conversational traffic management to independent variable tracking. In oral ngram 3 massage.txt, the token is continuously bound to real-time speech-pacing markers: *“massage okay so,” “massage all right,” “massage and then,”* and *“massage let s”*. These bundles function as conversational boundary markers, stepping listeners through an unedited sequence of spoken thoughts.

Conversely, *written ngram 3 massage.txt* showcases *massage* as an objectified scientific variable. The dominant bundles include *“massage therapy is”* (Freq: 23, Range: 7), *“massage therapy for”* (Freq: 17, Range: 6), *“massage along with”* (Freq: 13, Range: 2), and *“massage on constipation”* (Freq: 9, Range: 2).

Crucially, the compound bundle *“massage therapy and bodywork”* (and its variant *“massage and bodywork”*) appears prominently across both data files. Sociolinguistically, this fixed binomial expression acts as a professional "shield." Because the lone word "massage" carries historical, stigmatized associations with illicit sexual services, the deliberate, formulaic addition of "therapy" and "bodywork" serves as an institutional shield, legally and socially validating the clinical nature of the work.

Spatial Deixis vs. Conceptual Nominalization: The "Back" Token

The behavior of the token *back* showcases a complete structural realignment. In *oral ngram 4 back.txt*, the n-grams are highly frequent, spatial, and anchored to an immediate human body: *“back down the back”* (Freq: 4), *“back of the hand”* (Freq: 4), *“back of the neck”* (Freq: 3), and *“back up to the”* (Freq: 3). The speaker uses these bundles to build a physical roadmap in shared space.

In *written ngram 4 back.txt*, the frequency of every single n-gram drops to a flat baseline of 1. Written research does not look at an immediate, physical body in a room; instead, it translates anatomy into abstract data points. The written bundles convert spatial directives into clinical classifications: *“back and cervical region,” “back external oblique figure,”* and *“back pain associated with”*. The interactive, physical language of the oral register is entirely replaced by nominalized pathologies.

Methodological Agency and Passive Constructions: The Behavior of by and were

The written clinical corpus relies systematically on passive voice and agent-backgrounding to project scientific objectivity. A comparative keyness analysis against the 60,000-word COCA reference corpus validates this structural preference mathematically, identifying two tokens—*were* and *by*—as complementary markers of this detached, institutional register.

The token *were* achieves an immense keyness score of 2087.417 within the written subcorpus, establishing it as a dominant process marker. Conversely, inside the oral instructional subcorpus, *were* drops to a baseline keyness score of 134.885, where active first-person frameworks (*we're going to*, *i'm going to*) dominate instructional discourse instead.

A micro-analysis of the target text files reveals that passive voice in clinical writing is most frequently realized through this auxiliary *were*, which backgrounds human agency entirely without specifying an actor. Analysis of the data file *written ngram 3 were.txt* reveals that the highest-frequency bundles position *were* as the sole process marker, with no explicit agent:

N-gram	Rank	Freq	Range
were used in	1	6	5
were collected at	2	3	3
were rated as	2	3	1
were randomly assigned	5	2	1
were observed for	5	2	1

These bundles perform classic agent-defocusing functions common to experimental and clinical research reporting. The intervention, data collection, and outcome assessment are presented as procedural facts without human intervention. Whereas "*by*-phrases" retain a trace of agency, "*were"* constructions eliminate the agent entirely, producing a purely transactional, process-oriented register—representing a radical text environment featuring action without an actor.

The preposition "*by"* functions as a vital structural companion to this style, returning a massive keyness score of 1781.773 within the written reference subcorpus. This token was explicitly isolated from broader prepositional parsing due to its unique structural function in establishing empirical authority and managing process via two distinct patterns in *written ngram 3 by.txt*:

- **Attribution of Research:** "by the researchers" (Rank 1, Freq: 5), "by cohen and," and "by seyed rasouli".

- **Methodological Agency:** “by measuring the” (Freq: 3), “by modifying intra,” and “by examiner using”.

While *by* remains statistically significant in the oral corpus due to general speech distribution ($G^2 = 417.728$), its target usage shifts completely away from abstract agent-defocusing. Together, the *were* and *by* keyness surges provide definitive empirical verification of Biber’s Dimension 5 (Abstract vs. Non-Abstract Style). Written clinical register systematically uses these passive agentive frames to background human subjectivity. Rather than declaring "We massaged the client and measured their symptoms," the register strips away personal pronouns to emphasize objective, replicable methodology, highlighting parameters adjusted “by modifying intra” or evaluated “by the researchers”.

Discussion & Pedagogical Implementation

From Physical Anatomy to Financial Transaction: The "Welcome" Shift and Institutional Border Tracking

Consequently, the pedagogical focus was pivoted away from body geography and toward the register of professional-client transactions. Within this business sub-register, the learner struggled to articulate institutional policy without sounding overly confrontational or colloquially weak. To address this, the tutoring targeted prepositional-bound particles that modify core verbs, which radically alter a word's semantic environment. While the token welcome yielded a raw frequency of only 1 inside both individual subcorpora, a comparative keyness filtration against the COCA reference baseline proved its presence was far from anomalous. In both text environments, welcome returned an identical Log-Likelihood score of 18.246 and a positive effect size of 0.667. In corpus linguistics, an absolute log-likelihood threshold above 15.13 indicates extreme statistical significance at the $p < 0.0001$ level. This identical statistical fingerprint visually exposes the precise points where academic and regulatory institutions mark their professional territory. Concordance tracking shows that in the oral subcorpus, the token commands the entry into the educational training domain ("welcome back to Circadia University"). In the written subcorpus, it marks the formal boundary of professional association regulation via a citation trace ("Welcome to MANZ", referencing Massage New Zealand). This contrast reveals an important domain limitation: the statistical scarcity highlights a structural constraint of instructional video tutorials and formal clinical trials, which routinely bypass raw, unedited client-intake scenarios. Yet, the token's massive statistical keyness validates its immense communicative priority. In casual oral discourse, welcome functions as an interpersonal greeting ("You are welcome here"). However, when bound to a specific prepositional frame within a business policy statement, its semantic environment shifts from an informal greeting to an active, institutional regulation regarding payment acceptance (e.g., "We welcome payments by cash or card only; balances must be settled upon arrival"). Explicitly teaching the learner how particles and prepositional frames transform basic verbs allowed them to shift out of an unedited oral register and confidently command a formal, professional persona.

Deploying Academic Shields and Managing Client Boundaries

Crucially, this register awareness was weaponized pedagogically to help the learner navigate the unique sociolinguistic pressures and historical stigma attached to the industry. By introducing the fixed binomial expression "massage therapy and bodywork" as an academic and institutional shield, the tutor coached the student on how to establish firm professional and legal boundaries with clients and third-parties. In roleplaying exercises addressing ambiguous or potentially boundary-crossing client language, the learner was trained to transition from a defensive oral posture ("I don't do that kind of massage here") to a shielded, institutional register: "My practice strictly adheres to the scope of massage therapy and bodywork as defined by clinical and professional guidelines". This deliberate linguistic scaffolding transformed formulaic corpus bundles into an authoritative protective mechanism, empowering the student to claim professional legitimacy and safely enforce commercial and personal boundaries.

Inductive Scaffolding of the Passive Voice and "By"-Phrases

Because the corpus data highlighted the critical role of passive constructions and agentive by-phrases in establishing empirical authority, mastering these structures was deemed vital for the learner's clinical charting and SOAP note literacy. However, rather than utilizing deductive, abstract grammar drills—which risk inducing cognitive anxiety and functioning like an instructional "screamer video" for a developmental learner—the tutoring intervention deployed an inductive, text-driven reading activity. The learner was presented with authentic, short target excerpts from the written reference corpus (such as the abdominal massage protocols for chronic constipation) to act as a linguistic detective. Through guided discovery questions (e.g., "Who is performing the action in this sentence?" and "Why is the pronoun 'I' omitted?") the student naturally observed how bundles like "were collected at" or "by modifying intra" serve to background human subjectivity and foreground an objective, replicable clinical process. Once the student demystified these structures as functional professional tools rather than arbitrary academic hurdles, they were given grammatical scaffolding templates. This allowed them to successfully convert active student-talk ("I pressed on the lower back and found tight spots") into standard clinical register ("Palpation of the posterior myofascial chain was executed, by modifying intra-tissue pressure, to evaluate trigger point sensitivity").

Evaluating the Tutoring Intervention

Teaching the learner to recognize multi-word lexical bundles rather than isolated vocabulary words proved highly effective. By highlighting how written clinical texts use "massage therapy and bodywork" to project legitimacy, and how business transactions and clinical charting rely on passive frames and fixed verbal structures, the learner developed a functional, dual-register awareness. They learned that professional competence requires a split linguistic identity: using soft, involved mitigators at the massage table, and switching to dense, policy-driven structural bundles at the reception desk and in medical charts

Curriculum Scope and Domain Limitations

During the implementation phase, a critical boundary in the corpus architecture became apparent regarding independent business operations. While the tutoring intervention successfully mastered and deployed technical knowledge—such as lymphatic drainage pathways, foundational Swedish strokes, and standard clinical etiquette—it lacked the capacity to analyze or instruct on independent business structures.

For a therapist running their own business, navigating commercial legalities, marketing, and financial structures requires a vastly extended corpus. This domain of knowledge was recognized as being outside the initial scope of the investigation. Consequently, the intervention must be evaluated as a technical and clinical training aid rather than a comprehensive business-readiness tool. Future iterations would require an integrated secondary corpus dedicated to healthcare entrepreneurship to bridge this gap.

Data Preprocessing and Token Filtering

To optimize the model's focus on core therapeutic concepts, certain tokens were omitted during the corpus processing phase. It is critical to clarify that this omission was not systematic or indicative of an underlying linguistic pattern within the source material. Instead, tokens were filtered out on a purely practical basis. Textual and structural elements that did not align with the investigation's immediate scope were treated as noise and ignored to streamline data processing and maximize the relevance of the tutor's feedback loops.

Conclusion

This corpus-based case study demonstrates that register variation between oral and written massage therapy discourses is defined by deep structural and sociological realities. Oral instruction utilizes verbal mitigators (just) and spatial deixis (back) to guide real-time physical touch. Written clinical and business registers deploy compound nominal shields (massage therapy and bodywork), passive agent-backgrounding (by, were), and specialized prepositional verb frames (welcome) to assert professional legitimacy, manage stigma, and enforce commercial policies. For educators and tutors, these findings underline a critical pedagogical lesson: true vocational and clinical literacy cannot be achieved by teaching vocabulary lists in isolation. Learners must be explicitly trained to command the multi-word structural bundles that govern both the clinical lab and the business front office, ensuring their professional survival and authority. The complete corpus—including all oral and written transcripts as well as the n-gram and KWIC analysis files used in this study—is openly available for replication and further research at Zenodo: https://doi.org/10.5281/zenodo.20535483

Statement on the Use of Artificial Intelligence

The methodological framework of this study involved the use of Gemini (Google) as an advanced technological assistant. Specifically, the AI provided workflow optimization protocols for processing textual data within AntConc, including advice on tokenization settings and advanced search queries. Furthermore, Gemini assisted in organizing the preliminary data patterns extracted from the corpus. The AI was not used for data fabrication or text generation; the critical evaluation, corpus curation, and final synthesis of the findings were executed solely by the human researcher.

References

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Biber, D., Johansson, S., Leech, G., Conrad, S., & Finegan, E. (1999). *Longman grammar of spoken and written English*. Pearson.

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Halliday, M. A. K., & Matthiessen, C. M. (2014). *An introduction to functional grammar* (4th ed.). Routledge.

O'Keeffe, A., McCarthy, M., & Carter, R. (2007). *From corpus to classroom: Language use and language teaching*. Cambridge University Press.

Oral Register Annex

Below is the structured reference table for the oral register media data utilized within the corpus:

Source Title	**Quotation (Illustrative Example)**	**Key Curricular Focus**	**Reference URL**
Benefits of a full-body lymphatic massage	*"We're going to go ahead and make sure we clear that so going back we're going to go from below the ear down the neck again..."*	Sequential node stimulation, deep abdominal breathing, and fluid clearance protocols	https://www.youtube.com/watch?v=BFiv-zj8oog&t=2s

Circadia University Body Treatment: Dry Brushing Guide	*"We're going to go ahead and start our dry brushing. With dry brushing, you want to make sure that you're going to go up towards the heart."*	Exfoliation patterns for limbs/torso, circulatory stimulation, and professional "diaper drape" client modesty	https://www.youtube.com/watch?v=7TBf5MCR5g8
Effleurage and Petrissage to the Back	*"The key function of this effleurage technique is to apply that massage medium so we want to try and get the medium covered over the whole of*	Practical execution of double-handed effleurage, wringing, and skin rolling techniques	https://www.youtube.com/watch?v=ycJEUQw79bs&t=75s

	that wider back area..."		
Massage Basics: Anatomy Overview Pt 1	*"Homeostasis is the body's capacity to maintain relative consistency in its internal environment within very narrow ranges of change..."*	Directional terminology, homeostatic balance, tissue composition, and cellular anatomy	https://www.youtube.com/watch?v=sIQmgHju2HU
Massage Therapy Dos and Don'ts	*"What most people do to hurt themselves is overusing tools. If you're going to use them—even*	Injury mitigation for therapists, structural safety, tool use, and localized muscle stripping	https://www.youtube.com/watch?v=eetDGFjEnAE

	foam rollers—people want to foam roll every day."		
Nervous System Reset: Self foot reflexology massage	*"Reflexology is not a substitute for medical advice or treatment. So, please always consult a qualified health care professional..."*	Application of reflexology maps, zone therapy, manual ankle clearing, and relaxation protocols	https://www.yo utube.com/wat ch?v=xZFo0iJ zFkc
PROS & CONS of a Massage Therapy Career	*"The very first and main challenge to becoming a new massage therapist especially right*	Professional practice realities, income variability, client acquisition, and career sustainability	https://www.yo utube.com/wat ch?v=gC4nnU FA07Q

	out of school is that you will not earn a consistent income..."		
The FULL GUIDE to ALL MASSAGE STYLES!	*"Deep tissue is a very broad bucket and can include deep Swedish and really kind of covers the gamut for anything that essentially applies more pressure..."*	Comparative analysis of Swedish, Deep Tissue, Thai, Sports, and Myofascial modalities	https://www.youtube.com/watch?v=Gd6k8IqzuV0
Top Massage Etiquette Tips: What Your	*"When receiving a massage, it's essential to be*	Client/therapist boundaries, hygiene standards,	https://www.youtube.com/wat

Therapist Wishes You Knew	*mindful of proper etiquette to ensure a comfortable and respectful experience for both the client and the therapist."*	professional conduct, and contraindications	ch?v=3FwjrfHt3O8

Written Register Annex

Below is the structured reference table for the written register media data utilized within the corpus:

APA Reference (with DOI/URL)	Quotation (Illustrative Example)	Key Curricular / Analytical Focus

Arovan, N. I., & Maulana, G. F. (2026). The Effect of Self-Massage on Immune Function, Stress, and Quality of Life in Young Adults: A Pilot Randomized Control Trial. *International Journal of Therapeutic Massage & Bodywork*, 19(1), 6-17. https://doi.org/10.3822/ijtmb.v19i1.1141	"Self-massage enhances immune function and perceived general health in young adults, offering an effective approach."	Demonstrates passive agent-backgrounding (*were assigned*, *were collected*) and nominalized clinical outcomes in experimental research reporting.
Baskwill, A. (2026). Shaping Understanding: The Role of Professional Language in Massage Therapy. *International Journal of Therapeutic Massage & Bodywork*, 19(1), 1-5. https://doi.org/10.3822/ijtmb.v19i1.1459	"To the public, it may be unclear whether massage therapy is a health service, a wellness experience, or a form of complementary care."	Explicitly addresses professional register variation and the sociolinguistic stakes of inconsistent terminology across clinical, wellness, and public discourses.
Crawford, C., Boyd, C., Paat, C. F., Price, A., Xenakis, L., Yang, E., Zhang, W., & Evidence for Massage Therapy (EMT) Working Group (2016). The Impact of Massage Therapy on Function in Pain Populations—A Systematic Review and Meta-Analysis of Randomized Controlled Trials: Part I, Patients Experiencing Pain in the General Population. *Pain Medicine*, 17(7), 1353-1375.	"Results demonstrate massage therapy effectively treats pain compared to sham, no treatment, and active comparators."	Provides dense examples of methodological agency via by-phrases (*by the EMT Working Group*, *by measuring*) and hedged clinical claims in evidence synthesis.

https://doi.org/10.1093/pm/pnw099		
Fekri, Z., Aghebati, N., Sadeghi, T., & Farzadfard, M. T. (2021). The effects of abdominal "I LOV U" massage along with lifestyle training on constipation and distension in the elderly with stroke. *Complementary Therapies in Medicine*, 57, Article 102665. https://doi.org/10.1016/j.ctim.2021.102665	"The abdominal massage along with lifestyle training could improve constipation and distension and also increase food intake tolerance in the elderly patients with stroke."	Illustrates nominalized intervention protocols (*constipation improvement time*, *food tolerance rate*) and cautious clinical language (*could improve*, *may be reasons*).
Jensen, A. M. (2026). A Mindbody Approach to Long-standing Fatigue: A Case Report on Emotional Somatic Release Using HeartSpeak Lite for BodyWork. *International Journal of Therapeutic Massage & Bodywork*, 19(1), 86-91. https://doi.org/10.3822/ijtmb.v19i1.1349	"Fatigue that persists over time... often reflects a complex interplay of physiological, emotional, and lifestyle factors."	Exemplifies biopsychosocial register blending clinical terminology with mindbody and emotional somatic vocabulary (*interoception*, *emotional regulation*).
Kalyoncuo, S. (2026). Constipation management and abdominal massage in the elderly. *Sağlık Akademisi Kastamonu*, 11(1), 26-32. https://www.researchgate.net/publication/404465846_Constipation_Management_and_Abdominal_Massage_in_the_Elderly	"Abdominal massage is a non-invasive, safe, and well-tolerated intervention that does not cause pain or significant side effects."	Provides examples of clinical safety register (*non-invasive*, *well-tolerated*) and institutional language common in nursing and geriatric care literatures.

Mak, S., Allen, J., Begashaw, M., Miake-Lye, I., Beroes-Severin, B., De Vries, G., Lawson, E., & Shekelle, P. G. (2024). Use of massage therapy for pain, 2018-2023: A systematic review. *JAMA Network Open*, 7(7), Article e2422259. https://doi.org/10.1001/jamanetworkopen.2024.22259	"Massage therapy is a popular treatment that has been advocated for dozens of painful adult health conditions and has a large evidence base."	Demonstrates high-impact clinical register with cautious evidence language (*moderate-certainty evidence*, *low or very low certainty*) and passive constructions.
Malone, D. (2026, May 1). How NCAA basketball teams are using massage therapy. *American Massage Therapy Association*. https://www.amtamassage.org/publications/massage-therapy-journal/massage-elite-college-athletes/	"Our use of massage therapy has evolved significantly in recent years, both in how intentionally it's applied and how fully it's integrated."	Illustrates professional practice register in sports medicine, with first-person institutional voice (*we use*, *our program*) and applied clinical language.
Morgan, V., Smith, J., & Smith, D. (2026). Beyond the Barriers: Alternative Payment Schemes may make Massage Therapy more Affordable. *International Journal of Therapeutic Massage & Bodywork*, 19(1), 47-56. https://doi.org/10.3822/ijtmb.v19i1.1133	"Massage therapy is an increasingly utilized user-pays service... It is likely financially inaccessible to many and is generally not subsidized by the government."	Provides the business and financial register absent from the initial corpus, including economic vocabulary (*installment schemes*, *affordability*, *user-pays*).

Welling, A., Kage, V., Patil, A., Pereira, P., & Pujari, N. (2026). Prevalence of Posterior Myofascial Chain Tightness and Presence of Myofascial Trigger Points in Subjects with Cervicogenic Headache. *International Journal of Therapeutic Massage & Bodywork*, 19(1), 57-70. https://doi.org/10.3822/ijtmb.v19i1.1297	"Tightness in the lower back muscles can also contribute to pelvic misalignment, which, in turn, affects the cervical spine and exacerbates CGH symptoms."	Demonstrates anatomical and clinical register with causal chain language (*contributes to*, *affects*, *exacerbates*) and measurement terminology (*goniometer*, *pressure algometer*).

The complete corpus files, including all n-gram and KWIC exports, are archived at Zenodo with DOI: 10.5281/zenodo.20535483